

Proposed ACGME Common Program Requirements: What They Mean for Emergency Medicine

REVIEW, IMPACT, AND THE USF EMERGENCY MEDICINE REBUTTAL

Jason W. Wilson, MD, PhD, Professor and Chair, Department of Emergency Medicine | October 2026



UNIVERSITY of
SOUTH FLORIDA

Morsani College of Medicine
Emergency Medicine

Two separate processes, one comment deadline

	EM Program Requirements	Common Program Requirements (CPR)
Status	Revised and approved; effective 2028	Proposed major revision posted Sept 8, 2026
Scope	Emergency Medicine only (EM-RRC)	All residencies; RC authority defined by brackets
Comment	Closed	Open through October 22, 2026
Decision	Done	ACGME Board, June 2027; effective July 1, 2028
Framing		Presented as a “43 percent reduction” in the number of requirements

In several areas the proposed CPRs delete existing EM-specific requirements and remove or narrow EM-RRC authority to restore them.

We must respond to the proposed CPR.

Three changes happening together

1

Broader common standards

Common requirements become more general: outcome language turns into process language.

2

Deletion of specialty-specific requirements

Existing EM requirements in affected sections are removed on implementation.

3

Removal or narrowing of Review Committee authority

The EM-RRC can no longer further specify in those sections, so it cannot put the standards back.

Programs can exceed ACGME minimums, but local program expectations are not specialty-wide accreditation standards. Institutional resources for programs are usually guided by ACGME requirements, so without the requirement the ability to exceed it may never be realized.



Six proposed changes and how they hit Emergency Medicine

1. Core faculty: the 1:5 ceiling

	Current 2028 EM requirement	Proposed CPR	Impact on EM
Ratio	At least 1 core faculty member per 3 residents (EM §2.11.b)	RC may require no more than 1 core faculty per 5 residents (CPR §2.25 bracket)	EM-RRC cannot keep 1:3. USF EM required core faculty: 13 → 8
Protected time	RC may specify dedicated time for core faculty non-clinical work	RC “may” still specify time or role (§2.25.a) — optional	Fewer faculty with protected time for didactics, mentorship, assessment
Stated rationale	—	Reduce variability across specialties; lower barrier for small and under-resourced programs	An argument about cost, not education

Reducing core faculty also reduces the protected time available to junior faculty trying to build academic careers — further harming the future of academic EM and the advancement of the science.

2. Program director qualifications and continuity

	Current 2028 EM requirement	Proposed CPR	Impact on EM
Certification	Current ABEM or AOBEM; RRC-EM accepts only ABMS/AOA (EM §2.5)	Specialty certification “generally”; non-eligible PD may be approved by GMEC (§2.12); RC authority to require ABMS/AOA deleted	Opens EM residency leadership to a PD not certified in EM
Experience	≥3 years educational/administrative experience	<3 years allowed with a documented mentoring plan (§2.11.a)	Less experienced leadership
Approval	RC final approval of PD (§2.8.a)	GMEC approves; RC approval deleted	Specialty loses its check on PD selection
Continuity	Program must demonstrate PD retention (§2.9)	Deleted	Turnover warning sign removed

3. Physician faculty qualifications and supervision

	Current 2028 EM requirement	Proposed CPR	Impact on EM
Certification	ABEM/AOBEM or qualifications acceptable to RRC-EM (EM §2.10)	Specialty board certification; non-eligible faculty may receive GMEC approval (§2.24); RC authority to accept alternatives deleted	GMEC, not the EM-RRC, decides who may teach EM residents
Supervision	ED supervision by physicians who completed an EM residency; defined pediatric and low-acuity exceptions	Specialty supervision rules “should include flexibility” for supervision by qualified faculty in other specialties (§2.22 bracket)	Cross-specialty supervision beyond today’s exceptions

Rationale offered: relief for small programs with limited faculty.

4. Patient care, procedures, medical knowledge

	Current 2028 EM requirement	Proposed CPR	Impact on EM
Standard	“Residents must demonstrate...”	“The program must provide instruction in and assessment of...” (§4.4–4.6)	Outcome standard becomes a process standard
Specialty content	EM-specific patient care, clinical experience, procedural, and knowledge expectations (EM §4.4–4.6, §4.11)	Existing specialty-specific requirements removed; RC authority to further specify §4.4, 4.5, 4.6 deleted	EM-RRC loses the ability to define the essential competencies of an emergency physician
Sub-competencies	RC may add sub-competencies (PBLI, ICS, SBP)	Sub-competency lists consolidated; RC authority to add deleted (§4.7–4.9)	Less specialty-specific detail; AI language added (a positive)

5. Scholarship

	Current 2028 EM requirement	Proposed CPR	Impact on EM
Faculty	Collective accomplishment in ≥ 3 scholarly domains plus dissemination including peer-reviewed publication (EM §4.13–4.14)	Each core/required faculty: ≥ 1 qualifying activity every 3 years; a didactic presentation qualifies (§4.18.a)	Floor drops to one lecture every three years
Peer review	RC may require peer-reviewed publication	RC may no longer require peer-reviewed publication for faculty (§4.18.c)	Expectation to generate and disseminate new knowledge falls
Residents	RC may specify resident scholarship	≥ 1 activity before graduation; RC authority deleted (§4.18.b, §4.19)	Resident scholarship becomes a checkbox

6. Work hours: the deleted bracket

	Current 2028 EM requirement	Proposed CPR	Impact on EM
Weekly hours	≤60 scheduled ED hours; ≤72 total (EM §6.17)	≤80 hours averaged over up to 4 weeks (§6.29)	EM limits deleted
Shift length	≤12 continuous hours with equivalent time off	≤24 hours + 4 for transitions (§6.33)	12-hour shift limit deleted
Days off	1 in 7 without averaging; ≤6 consecutive days	1 in 7 averaged over up to 4 weeks (§6.32)	Non-averaged day off deleted
Time between shifts	Equivalent time off between scheduled ED work periods (EM §6.17)	Common 8-hours-off rule (§6.30) deleted; EM rule deleted	No minimum turnaround between shifts anywhere in the requirements
RC authority	RC may further specify §6.29–6.39	Bracket deleted (under §6.39.a)	EM-RRC cannot restore any of the above

Other notable proposed changes

Change	Proposed	Our position
Board certification (§5.15–5.16)	≥90% ultimate certification within 7 years; first-time standard becomes 2 of last 3 exams	Support with modifications. 2-of-3 forgives a failed cohort
Environment of inquiry (§4.17)	New explicit requirement for critical appraisal and lifelong learning	Support
Technology and AI (§4.3, §4.9)	Ethical use of technology including AI written into two competencies	Support
Leadership support (§2.14, §2.26.a)	PD and coordinator support tied to program size and complexity	Support
Self-Study (§5.14.h)	Deletes the 10-year Self-Study (program-wide self-assessment submitted to the DIO before the 10-Year Site Visit)	Neutral — burden reduction; Annual Program Evaluation remains
Post-call rest (§6.31); every-third-night call (§6.38)	Rest after 24-hour in-house call 14 → 12 hours; call-frequency limit deleted	24-hour call only (off-service rotations); does not touch ED shifts
Time between shifts (§6.30; EM §6.17)	Common 8-hours-off rule and EM equivalent-time-off rule both deleted	This one does affect EM turnarounds — see slide 10

The USF Emergency Medicine rebuttal

Expansion has already produced a tiered specialty

Rapid expansion of EM programs outside academic institutions (varying patient populations, hospital environments, faculty talent, and support) has created a heterogeneous training environment less likely to develop the faculty who advance the field; graduates of accredited programs do not leave with equal ability.

Program type (n)	Faculty per program	No academic rank	Assoc./full professor	h-index = 0	h-index ≥10
Academic (141)	72	8%	32%	38%	16%
Corporate-affiliated (76)	19	58%	15%	55%	7%
Community-based (82)	19	72%	7%	56%	4%

Source: USF EM Faculty Census, Sept 2026 (304 ACGME EM programs; 12,911 faculty; Scopus h-index).

Academic = AAU/NIH-ranked, Vizient, or university-based. Protected time not captured; non-academic programs often fund only the required FTE.

The proposed CPRs water the requirements down further. Departments with resources will set higher internal expectations but programs without them will meet the new minimum — a more tiered distribution of programs.

The ACGME's role should be to protect the training of residents and the future of academic medicine for patients and trainees. These changes will produce inequities in training quality.

What the ACGME should be for

Ensure standards

Standards across programs that produce highly trained physicians for our communities. Easier to meet is not the same as better.

Let specialties adapt

Set the floor and let each specialty, through its Review Committee, adapt, modify, and adopt the requirements that fit its field. EM should be able to keep 60 hours per week, keep program directors as board-certified experts in EM education, and keep faculty ratios at 1:3.

Balance corporate influence

Corporate medicine and private equity have driven EM GME growth and see less regulation and lower cost as favorable. The ACGME should be the counterweight.



Core faculty is where academic EM develops people

1:3

EM today

1:5

proposed ceiling

13 → 8

USF EM required core faculty

Core faculty status gives protected time to physicians who may want academic careers. With mentorship, that grows into a lower clinical footprint and the space to develop the future.

Corporate programs have dominated EM GME expansion for ten years (smaller academic groups, less progression through rank, lower scholarly output, less funded research, and often only the required FTE).

A 1:5 ceiling means fewer dedicated faculty for didactics and mentorship, fewer physicians who develop as academicians, and divestment from academic EM.

Ask: retain EM-RRC authority to set the ratio the specialty needs; no ceiling may override an existing, more rigorous specialty standard.



Leadership and faculty must be EM experts

Program director

Must be board certified in the specialty (ABEM/AOBEM).

Three years is not a magic number, but it is a reasonable floor.

If a program cannot field an expert-level program director, it should not be a program.

Ask: keep 2.11–2.13 as the EM-RRC defines them; no GMEC pathway for non-eligible PDs.

Physician faculty

Must be scholars in EM: peer-reviewed publication, research, and service (committees).

EM residents in the ED are supervised by EM-residency-trained physicians; existing pediatric and low-acuity exceptions already provide flexibility.

Ask: strike the “should include flexibility” bracket in 2.22; retain RC authority over faculty qualifications in 2.24.

Resident scholarship and sensible work hours

Resident training

Train every resident for a potential academic career and for lifelong learning, with program support for scholarship.

Require a scholarly product and participation in patient safety reviews and quality projects — integral to any career, academic or community.

Ask: strengthen 4.18.b; retain RC authority in 4.19 and 4.4–4.6.

Work hours

An 80-hour ceiling averaged over four weeks is acceptable; no EM program schedules near it when typical ED hours are closer to 45, and block flexibility helps.

The problem is deleting RC authority to further specify 6.29–6.39, which removes EM's 60-hour week, 12-hour shift, and non-averaged day off.

Ask: Keep 60 hours per week in EM.



What we are asking the ACGME to change

CPR §	Proposed	Ask
2.25	Core faculty ratio capped at 1:5	Retain RC authority to set the ratio; no ceiling overrides an existing stricter standard
2.11–2.13	3-year minimum replaced by mentoring; GMEC path for non-eligible PD; RC authority deleted	Keep the 3-year floor and specialty certification; keep RC authority and approval
2.22, 2.24	Cross-specialty supervision “flexibility”; GMEC path for faculty	Strike the bracket; retain RC authority over faculty qualifications
4.4–4.6	Outcome to process; RC authority deleted	Restore “residents must demonstrate” and RC authority
4.18–4.19	One activity per 3 years; no peer-review requirement; RC authority deleted	Keep EM’s three-domain standard; let RC require peer-reviewed publication
6.39.a bracket	RC authority over 6.29–6.39 deleted	Restore it; keep the 80-hour ceiling as a ceiling

What to do before October 22

Every faculty member and resident

Submit an individual comment at acgme.org → Review and Comment → Common Program Requirements (Residency).

Use the USF EM page-by-page guide (Box → Claude outputs → ACGME CPR Response Oct 2026). About 30 minutes.

Personalize at least one comment; identical text is counted once.

Report submissions to the department so we can track our count.



USFHealth